

Performance Work Statement for Teleradiology Physician Services

1. GENERAL:

- 1.1. Services Provided: The contractor shall furnish board certified or board eligible Radiology Physician services in accordance with the specifications contained herein to the beneficiaries of the Department of Veterans Affairs (VA) and the New Mexico VA Health Care System (NMVAHCS). The VA Radiology Services follow the standards & guidelines set forth by American College of Radiology (ACR) [Practice Parameters and Technical Standards | American College of Radiology \(acr.org\)](#). A contractor providing teleradiology services shall provide services consistent with The Joint Commission (TJC) and that meet or exceed VA and the American College of Radiology Guidelines as defined in this document.
- 1.2. Place of Performance: Services shall be provided off-site at the Contractor's location.
- 1.3. Authority: Title 38 United States Code (U.S.C.) § 8153 – Sharing of Health Care Resources (HCR), Federal Acquisition Regulation (FAR) Part 12 - Acquisition of Commercial Products and Commercial Services- in conjunction with FAR Part 15 – Contracting by Negotiation.
- 1.4. Policy/Directives/Handbooks. The contractor shall be subject to the following policies, including any subsequent updates during the period of performance. The policies listed below can be accessed electronically at the following: [VA Publications](#) and [VHA Publications](#).
 - 1.4.1. VA Directive 1663: Health Care Resources (HCR) Contracting – Buying Title 38 U.S.C. 8153
 - 1.4.2. VHA Directive 1003.04: VHA Patient Advocacy
 - 1.4.3. VHA Directive 1065(1): Productivity and Staffing Guidance for Specialty Provider Group Practice
 - 1.4.4. VHA Directive 1088(1): Communicating Test Results to Providers and Patients
 - 1.4.5. VHA Directive 1100.18: Reporting and Responding to State Licensing Boards
 - 1.4.6. VHA Directive 1100.20(2): Credentialing of Health Care Providers
 - 1.4.7. VHA Directive 1100.21(1): Privileging
 - 1.4.8. VHA Directive 1417: Lung Cancer Screening
 - 1.4.9. VHA Directive 1605.01: Privacy and Release of Information
 - 1.4.10. VHA Directive 1907.01(1): VHA Health Information Management and Health Records
 - 1.4.11. VHA Directive 1907.08: Health Care Information Security Policy and Requirements
 - 1.4.12. VHA Handbook 1100.17: National Practitioner Data Bank (NPDB) Reports
 - 1.4.13. Privacy Act of 1974 (5 U.S.C. 552a) as amended:
http://www.justice.gov/oip/foia_updates/Vol_XVII_4/page2.htm
 - 1.4.14. 42 Code of Federal Regulations (CFR) Part 482 Conditions of Participation Hospitals: <https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-G/part-482>
- 1.5. Acronyms/Definitions: Terms used in this contract shall be interpreted as follows unless the context expressly requires a different construction and/or interpretation. In case of a conflict

in language between the Definitions and other sections of this contract, the language in this section shall govern.

- 1.5.1. ACGME: Accreditation Council for Graduate Medical Education
- 1.5.2. ACLS: Advanced Cardiac Life Support
- 1.5.3. BLS: Basic Life Support
- 1.5.4. CDC: Centers for Disease Control and Prevention
- 1.5.5. CEU: Certified Education Unit
- 1.5.6. Clinical Privileging: Clinical Privileging is the process by which a practitioner, licensed for independent practice; e.g., without supervision, direction, required sponsor, preceptor, mandatory collaboration, etc.; is permitted by law and the facility to practice independently, to provide specific medical or other patient care services within the scope of the individual's license, based upon the individual's clinical competence as determined by peer references, professional experience, health status, education, training and licensure. Clinical privileges must be facility-specific and provider-specific, and within available resources.
- 1.5.7. CME: Continuing Medical Education
- 1.5.8. CMS: Centers for Medicare and Medicaid Services
- 1.5.9. CO: Contracting Officer: The person executing this contract on behalf of the Government with the authority to enter into and administer contracts and make related determinations and findings.
- 1.5.10. COR: Contracting Officer's Representative: A person appointed by the CO to take necessary action to ensure the Contractor performs in accordance with and adheres to the specifications contained in the contract and to protect the interest of the Government. The COR shall report to the CO promptly any indication of non-compliance in order that appropriate action can be taken.
- 1.5.11. COS: Chief of Staff
- 1.5.12. CPARS: Contractor Performance Assessment Reporting System
- 1.5.13. Credentialing: Credentialing is the process of obtaining, verifying, and assessing the qualifications of a health care provider to provide care or services in or for the VA health care system. Credentials are documented evidence of licensure, education, training, experience, or other qualifications.
- 1.5.14. DEA: Drug Enforcement Agency
- 1.5.15. EHR: Electronic Health Record - electronic health record system used by the VA
- 1.5.16. FSMB: Federation of State Medical Boards
- 1.5.17. FTE: Full Time Equivalent. VA's definition for full time-working the equivalent of 80 hours every two weeks, 2,080 hours per year. In calculating FTE, any hours not worked on national holidays shall not be included.
- 1.5.18. HHS: Department of Health and Human Services
- 1.5.19. HIPAA: Health Insurance Portability and Accountability Act
- 1.5.20. ISO: Information Security Officer

- 1.5.21. Key Personnel: The individuals specified in this contract who are essential to work performance.
- 1.5.22. NPI: National Provider Identifier. NPI is a standard, unique 10-digit numeric identifier required by HIPAA. The Veterans Health Administration must use NPIs in all HIPAA-standard electronic transactions for individual (health care practitioners) and organizational entities (medical centers)
- 1.5.23. OPPE: Ongoing Provider Practice Evaluation
- 1.5.24. PACS: Picture Archival Communication System
- 1.5.25. POP: Period of Performance
- 1.5.26. PPD: Purified Protein Derivative
- 1.5.27. PWS: Performance Work Statement
- 1.5.28. QA/QI: Quality Assurance/Quality Improvement
- 1.5.29. QASP: Quality Assurance Surveillance Plan
- 1.5.30. QM/PI: Quality Management/Performance Improvement
- 1.5.31. VETPro: is VHA's mandatory credentialing software platform to document the credentialing of VHA health care providers. This system facilitates completion of a uniform, accurate, and complete credentials file.
- 1.5.32. VHA: Veterans' Health Administration
- 1.5.33. VISN: Veterans Integrated Services Network
- 1.5.34. VISTA: Veterans Information Systems Technology Architecture

2. QUALIFICATIONS:

2.1. Staff/Facility

- 2.1.1. License: Contract physician(s) assigned by the Contractor to perform the services covered by this contract shall have a current license to practice medicine in any State Territory, or Commonwealth of the United States or the District of Columbia where services are being performed. All licenses held by the key personnel working on this contract shall be full and unrestricted licenses. Information on state licensing in tele-radiology may be found at this site: [Teleradiology | American College of Radiology \(acr.org\)](http://Teleradiology | American College of Radiology (acr.org)). Contract physician(s) who have current, full and unrestricted licenses in one or more states, but who have, or ever had, a license restricted, suspended, revoked, voluntarily revoked, voluntarily surrendered pending action or denied upon application may be considered for the purpose of this contract only if the provider is successfully credentialed and privileged by NMVAHCS Credentialing and Privileging Office. Acceptance under this contract is contingent upon the provider meeting all clinical qualifications and being granted privileges by NMVAHCS.
- 2.1.2. Board Certification: All contract physician(s) shall be board certified or board eligible in Diagnostic Radiology Medicine. All continuing education courses required for maintaining certification must be kept up to date. Documentation verifying current certification shall be provided by the Contractor to the VA COR on an annual basis for each year of contract performance.

- 2.1.3. Credentialing and Privileging: Credentialing and privileging is to be done in accordance with the provisions of VHA Directive 1100.20(2) and VHA Directive 1100.21(1) referenced above. The Contractor is responsible to ensure that proposed physician(s) possesses the requisite credentials enabling the granting of privileges. No services shall be provided by any contract physician(s) prior to obtaining approval by the NMVAHCS Professional Standards Board, Medical Executive Board and Medical Center Director.
- 2.1.3.1. If a contract physician(s) is not credentialed and privileged or has credentials/privileges suspended or revoked, the Contractor shall furnish an acceptable substitute without any additional cost to the government.
- 2.1.3.2. When teleradiology services are being provided by contract physician(s) who direct, diagnose, or otherwise provide clinical treatment to a patient via a teleradiology link, the contract physician(s) must be appointed, credentialed, and privileged at the VA facility (facilities) which receive the teleradiology services (patient site), as well as at the site providing the services. The following requirements apply:
- 2.1.3.3. Contract licensed independent practitioners must be credentialed and privileged through the VA facility's Medical Staff Office in accordance Credentialing and Privileging Directives 1100.20(2) and 1100.21(1) and VHA Directive 1914(1).
- 2.1.3.4. Separately delineated privileges for teleradiology services must be requested and assigned in this process.
- 2.1.3.5. In general, the credentialing process should be started at least 60-90 days in advance of estimated entry on duty. The time required for this process may be expedited when patient care needs are urgent.
- 2.1.3.6. The VA's Medical Staff Office or Human Resources Management Service will provide an instruction packet containing forms and a detailed listing of required documentation, including curriculum vitae, current references, signed release of information from the candidate, and statement that the candidate does not have any physical or mental health condition that would adversely affect his/her ability to carry out the assigned duties, and additional credentialing forms. The candidate will also be required to make application through the VetPro internet-based process. The candidate will need to be enrolled in VetPro by the Credentialing Coordinator at the VA or by the Human Resources Management Service designee. The website address is: <https://fcp.vetpro.org/Admin/Login/Index>
- 2.1.3.7. It is the Contractor's responsibility to contact the NMVAHCS Radiology Department to request a package and instructions.
- 2.1.3.8. When contract physician(s) provide only teleconsultation by offering advice that supports care provided by VAs on site licensed independent providers, the Contractor must furnish VA with a copy of the contract physician(s) credentials, verification of credentials, and current clinical privileges. The contract physician(s) providing teleconsultation services do not have to be separately privileged at the VA facility.
- 2.1.3.9. The Contractor must furnish a copy of its policy and procedures for credentialing and privileging to the Contracting Officer and COR for review

to ensure these meet the VA's requirements for establishment of competence through these mechanisms.

- 2.1.4. Technical Proficiency - Contract physician(s) shall be technically proficient in the skills necessary to fulfill the government's requirements, including the ability to speak, understand, read and write English fluently. Contractor shall provide documents upon request of the CO/COR to verify current and ongoing competency, skills, certification and/or licensure related to the provision of care, treatment and/or services performed. Contractor shall provide verifiable evidence of all educational and training experiences including any gaps in educational history for all contract physician(s) and contract physician(s) shall be responsible for abiding by the Facility's Medical Staff By-Laws, rules, and regulations that govern medical staff behavior. See Section D - Attachments: D.1 – NMVAHCS Medical Staff By-Laws.
- 2.1.5. Continuing Medical Education (CME)/ Certified Education Unit (CEU) Requirements: Contractor shall provide the COR copies of current CMEs as required or requested by the Facility. Contract physician(s) registered or certified by national/medical associations shall continue to meet the minimum standards for CME to remain current. Contractor shall report CME hours to the credential's office for tracking. These documents are required for both privileging and re-privileging. Failure to provide shall result in loss of privileges for contract physician(s).
- 2.1.6. Training (VA MANDATORY): Contractor shall meet all VA educational requirements and mandatory course requirements defined herein; all training must be completed by the contract physician(s) as required by the VA.

Training	Frequency	Annual Hours
VA Privacy Policy and Security Training	Yearly	1 Hour
HIPAA Training	Yearly	1 Hour

- 2.1.7. National Provider Identifier (NPI): NPI is a standard, unique 10-digit numeric identifier required by HIPAA. The Veterans Health Administration must use NPIs in all HIPAA-standard electronic transactions for individual (health care practitioners) and organizational entities (medical facilities). The Contractor shall have or obtain appropriate NPI and if pertinent the Taxonomy Code confirmation notice issued by the Centers for Medicare and Medicaid Services (CMS) National Plan and Provider Enumeration System (NPPES) be provided to the Contracting Officer with the proposal.
- 2.1.8. Conflict of Interest: The Contractor and all contract physician(s) are responsible for identifying and communicating to the CO and COR conflicts of interest at the time of proposal and during the entirety of contract performance. At the time of proposal, the Contractor shall provide a statement which describes, in a concise manner, all relevant facts concerning any past, present, or currently planned interest (financial, contractual, organizational, or otherwise) or actual or potential organizational conflicts of interest relating to the services to be provided. The Contractor shall also provide statements containing the same information for any identified consultants or subcontractors who shall provide services. The Contractor must also provide relevant facts that show how it's organizational and/or management system or other actions would avoid or mitigate any actual or potential organizational conflicts of interest. These statements shall be in response to the VAAR provision 852.209-70

Organizational Conflicts of Interest. See Section D - Attachments: D.2 – VAAR provision 852.209-70: Organizational Conflicts of Interest.

2.1.9. Citizenship related Requirements:

2.1.9.1. The Contractor certifies that the Contractor shall comply with any and all legal provisions contained in the Immigration and Nationality Act of 1952, As Amended; its related laws and regulations that are enforced by Homeland Security, Immigration and Customs Enforcement and the U.S Department of Labor as these may relate to non-immigrant foreign nationals working under contract or subcontract for the Contractor while providing services to Department of Veterans Affairs patient referrals.

2.1.9.2. While performing services for the Department of Veterans Affairs, the Contractor shall not knowingly employ, contract, or subcontract with an illegal alien; foreign national non-immigrant who is in violation their status, as a result of their failure to maintain or comply with the terms and conditions of their admission into the United States. Additionally, the Contractor is required to comply with all “E-Verify” requirements consistent with “Executive Order 12989” and any related pertinent Amendments, as well as applicable Federal Acquisition Regulations.

2.1.9.3. If the Contractor fails to comply with any requirements outlined in the preceding paragraphs or its Agency regulations, the Department of Veterans Affairs may, at its discretion, require that the foreign national who failed to maintain their legal status in the United States or otherwise failed to comply with the requirements of the laws administered by Homeland Security, Immigration and Customs Enforcement and the U.S Department of Labor, shall be prohibited from working at the Contractor’s place of business that services Department of Veterans Affairs patient referrals; or other place where the Contractor provides services to veterans who have been referred by the Department of Veterans Affairs; and shall form the basis for termination of this contract for breach.

2.1.9.4. This certification concerns a matter within the jurisdiction of an agency of the United States and the making of a false, fictitious, or fraudulent certification may render the maker subject to prosecution under 18 U.S.C. 1001.

2.1.9.5. The Contractor agrees to obtain a similar certification from its subcontractors. The certification shall be made as part of the offerors response to the RFP using the subject attachment in Section D of the solicitation document. See Section D - Attachments: D.3 Contractor Certification Immigration and Nationality Act of 1952, As Amended.

2.1.10. Annual Office of Inspector General (OIG) Statement: In accordance with HIPAA and the Balanced Budget Act (BBA) of 1977, the Department of Health and Human Services (HHS) Office of Inspector General (OIG) has established a list of parties and entities excluded from Federal health care programs. Specifically, the listed parties and entities may not receive Federal Health Care program payments due to fraud and/or abuse of the Medicare and Medicaid programs.

2.1.10.1. Therefore, Contractor shall review the HHS OIG List of Excluded Individuals/Entities on the HHS OIG web site at <http://oig.hhs.gov/exclusions/index.asp> to ensure that the proposed

contract physician(s) are not listed. Contractor should note that any excluded individual or entity that submits a claim for reimbursement to a Federal health care program, or causes such a claim to be submitted, may be subject to a Civil Monetary Penalty (CMP) for each item or service furnished during a period that the person was excluded and may also be subject to treble damages for the amount claimed for each item or service. CMP's may also be imposed against the Contractor that employ or enter into contracts with excluded individuals to provide items or services to Federal program beneficiaries.

2.1.10.2. By submitting their proposal, the Contractor certifies that the HHS OIG List of Excluded Individuals/Entities has been reviewed and that the Contractors are and/or firm is not listed as of the date the offer/bid was signed.

- 2.2. Clinical/Professional Direction: The qualifications of Contractor personnel are subject to review by VA Medical Facility COS or his/her clinical designee and approval by the Medical Facility Director as provided in VHA Directive 1100.20(2) and VHA Directive 1100.21(1). Clinical/Professional direction of all clinical personnel covered by this contract for quality purposes will be provided by the Facility COS and/or the Chief of the Service or his designee. A clinical COR may be appointed, however, only the CO is authorized to consider any contract modification request and/or make changes to the contract during the administration of the resultant contract.
- 2.3. Non-Personal Healthcare Services: The parties agree that the Contractor and all contract physician(s) shall not be considered VA employees for any purpose.
- 2.4. Indemnification: The Contractor shall be liable for, and shall indemnify and hold harmless the Government against, all actions or claims for loss of or damage to property or the injury or death of persons, arising out of or resulting from the fault, negligence, or act or omission of the Contractor, its agents, or employees.
- 2.5. Prohibition Against Self-Referral: Contractor's physicians are prohibited from referring VA patients to contractor's or their own practice(s)
- 2.6. Inherent Government Functions: Contractor and Contract physician(s) shall not perform inherently governmental functions. This includes, but is not limited to, determination of agency policy, determination of Federal program priorities for budget requests, direction and control of government employees (outside a clinical context), selection or non-selection of individuals for Federal Government employment including the interviewing of individuals for employment, approval of position descriptions and performance standards for Federal employees, approving any contractual documents, approval of Federal licensing actions and inspections, and/or determination of budget policy, guidance, and strategy.
- 2.7. No Employee status: The Contractor shall be responsible for protecting Contract physician(s) furnishing services. To carry out this responsibility, the Contractor shall provide or certify that the following is provided for all their staff providing services under the resultant contract:
 - 2.7.1. Workers' compensation;
 - 2.7.2. Professional liability insurance;
 - 2.7.3. Health examinations;
 - 2.7.4. Income tax withholding; and

2.7.5. Social security payments

- 2.8. Tort Liability: The Federal Tort Claims Act does not cover Contractor or contract physician(s). When a Contractor or contract physician(s) has been identified as a provider in a tort claim, the Contractor shall be responsible for notifying their legal counsel and/or insurance carrier. Any settlement or judgment arising from a Contractor's (or contract physician(s)) action or non-action shall be the responsibility of the Contractor and/or insurance carrier.
- 2.9. Contingency Plan: Because continuity of care is an essential part of the Facility's medical services, The Contractor shall have a contingency plan in place to be utilized if the contract physician(s) leaves Contractor's employment or is unable to continue performance in accordance with the terms and conditions of the resulting contract.
- 2.9.1. Contractor shall submit a listing of proposed key personnel performing technical delivery of services in the following format:
- Provider Name: (available when contract is awarded)
- Title/Rank: (available when contract is awarded)
- Curriculum Vitae and qualification documents of each key personnel shall be submitted with the proposal.
- 2.10. Emergency Substitutions: During the first ninety (90) calendar days of performance, the Contractor shall make NO substitutions of key personnel unless the substitution is necessitated by illness, death or termination of employment. The Contractor shall notify the CO, in writing, within fifteen (15) calendar day (s) after the occurrence of any of these events and provide the information required below. After 90 days, the Contractor shall submit the information required below to the CO at least fifteen (15) days calendar days prior to making any permanent substitutions.
- 2.10.1. The Contractor shall provide a detailed explanation of the circumstances necessitating the proposed substitutions, complete resumes for the proposed substitutes, and any additional information requested by the CO. Proposed substitutes shall have comparable qualifications to those of the persons being replaced. The CO will notify the Contractor within fifteen (15) calendar days after receipt of all required information of the decision on the proposed substitutes. The contract will be modified to reflect any approved changes of key personnel.
- 2.10.2. For temporary substitutions where the key person shall not be reporting to work for three (3) consecutive workdays or more, the Contractor shall provide a qualified replacement for the key person. The substitute shall have comparable qualifications to the key person. Any period exceeding two weeks will require the procedure as stated above.

3. VA HOURS OF OPERATION/SCHEDULING:

- 3.1. Business Hours: The contractor shall be available Monday through Friday from 16:00 PM to 08:00 AM, and 24 hours per day on Saturdays, Sundays, and all Federal holidays. Transmission of images for interpretation may occur outside of these work schedule windows.
- 3.2. Contractor shall be available to the Facility for consultation and interpretation 24 hours a day, 7 days a week, 52 weeks a year for STAT, urgent, and routine exams as needed.

3.3. Federal Holidays: The following holidays are observed by the Department of Veterans Affairs:

- New Year's Day
- Martin Luther King's Birthday
- President's Day
- Memorial Day
- Juneteenth
- Independence Day
- Labor Day
- Columbus Day
- Veterans Day
- Thanksgiving Day
- Christmas Day
- Any day specifically declared to be a national holiday.

3.4. Continuity of services under contract:

- 3.4.1. Unless a state of emergency has been declared or clinics are otherwise cancelled by the NMVAHCS the Contractor shall provide the services specified under this contract.
- 3.4.2. Contractor is responsible for providing qualified personnel to read films. The VA must be informed at least fifteen (15) calendar days in advance if there will be a significant change in contractor's ability to perform to allow for planning of service coverage and workload distribution. Requirements as noted in attachment B will be provided to all Facilities and maintained by the contractor. (D.4)
- 3.4.3. The contractor shall maintain open and professional communication with members of the VISN and Facility Diagnostic Imaging staff. The contractor shall provide up-to-date points of contact to facilitate communication on technical and administrative issues including review of quality measures and the VA OPPE process. The Contractor will provide a schedule of radiologists with telephone numbers for consultation by VA technologists and referring physicians.
- 3.4.4. The contractor shall provide a single phone number with a list of individuals who could be contacted to immediately resolve any difficulties experienced of teleradiology services and STAT studies. The COR will assure up-to-date rosters of staff are provided to the Contractor.
- 3.4.5. Tele-communications:
- 3.4.5.1. The contractor will secure and maintain its communications link between the contractor's facility and the VAMCs' facilities at the contractor's expense. These expenses may be included in contractor's proposal as a separate item. The cost to establish and maintain this communication link may be provided, if the Government deems it is more cost effective to the Government.

- 3.4.5.2. In the event of telecommunications failure, the Government shall package, hard copy film or alternative media in Government-provided shipping containers, and ship to the contractor's facility; using Government provided pre-paid shipping documentation. The contractor's place of business must be within the confines of the continental United States.
- 3.4.5.3. The contractor shall make provisions for the proper maintenance and functioning of all teleradiology, and associated equipment, facilities and fixtures as may reasonably be required by Contract Physicians to perform services hereunder.

4. CONTRACTOR RESPONSIBILITIES:

- 4.1. Clinical Personnel Required: The Contractor shall provide contract physician(s) who are competent, qualified per this PWS and adequately trained to perform assigned duties.
- 4.2. Management and Supervision:
 - 4.2.1. The Contractor shall be responsible for supervising the daily services provided under this contract by the Contractor's staff.
 - 4.2.2. The Contractor shall have written policies and procedures regarding staff credentials and privileging.
 - 4.2.3. The Facility will provide to the Contractor policies, procedures and processes necessary to allow cooperative functioning between the agency and NMVAHCS. Updates and refreshers will be provided to the Contractor upon request and when policy procedures or process changes.
 - 4.2.4. The Contractor shall complete and maintain background investigations to insure that employees do not have a record of criminal offenses or substantiated incidents of patient abuse; and, if required to perform their duties, employees are properly licensed and insured to operate motor vehicles. A copy of the initiation of the background investigation as well as a completed background investigation shall be provided to the NMVAHCS.
- 4.3. Standards of Care: The contract physician(s)' care shall cover the range of Radiology services as would be provided in a civilian medical treatment facility and the standard of care shall be of a quality, meeting or exceedingly currently recognized national standards as established by:
 - 4.3.1. The American College of Radiology Guidelines: [Practice Parameters and Technical Standards | American College of Radiology \(acr.org\)](#)
 - 4.3.2. ACR Practice Parameter for Communication of Diagnostic Imaging Findings: [Practice Parameters and Technical Standards | American College of Radiology \(acr.org\)](#);
 - 4.3.3. VA Standards: All teleradiology interpretation shall begin immediately upon receipt. The contractor shall provide the VAMC with a final report of findings within sixty (60) minutes of complete receipt of exam images for STAT, four (4) hours for urgent

exams. Routine studies shall include a final report of findings and shall be provided to the VAMC by the Contractor within forty-eight (48) hours.

NMVAHCS Standards: See Section D – Attachments: D.5 – MCP 11-64(1): Critical Test Results and D.6 – MCP 11-81: Ordering and Reporting Test Results.

4.3.4. The professional standards of the Joint Commission (TJC):
http://www.jointcommission.org/standards_information/hap_requirements.aspx

4.3.5. The standards of the American Hospital Association (AHA):
<http://www.hpoe.org/resources?show=100&type=8> and;

4.3.6. The requirements contained in this PWS.

4.4. **MEDICAL RECORDS:**

4.4.1. Authorities: Contractor's physician(s) providing healthcare services to VA patients shall be considered as part of the Department Healthcare Activity and shall comply with the 5 U.S.C.552a (Privacy Act), 38 U.S.C. 5701 (Confidentiality of claimants records), 5 U.S.C. 552 (FOIA), 38 U.S.C. 5705 (Confidentiality of Medical Quality Assurance Records) 38 U.S.C. 7332 (Confidentiality of certain medical records), Title 5 U.S.C. § 522a (Records Maintained on Individuals) as well as 45 C.F.R. Parts 160, 162, and 164 (HIPAA).

4.4.2. HIPAA: This contract and its requirements meet exception in 45 CFR 164.502(e), and do not require a BAA in order for Covered Entity to disclose Protected Health Information to: a health care provider for treatment of VA patients. Based on this exception, a BAA is not required for this contract. Health records generated by this contract or provided to the Contractors by the VA are covered by the VA Privacy Act system of records entitled 'Patient Medical Records-VA' (24VA10A7). Contractor generated VA Patient records are the property of the VA and shall not be accessed, released, transferred, or destroyed except in accordance with applicable laws and regulations. Contractor shall ensure that all records pertaining to medical care and services provided to VA patients are captured in the VA electronic health record system as required by VA policy as discussed in 4.4.4.

4.4.3. Disclosure: Contractor's physician(s) may have access to patient medical records for the purpose of providing medical care and services to VA patients and performing services under the contract. VA authorizes the Contractor to discuss patient health information for coordination of care with community health care providers in compliance with VA regulations, HIPAA and VHA Directive 1605.01, Privacy and Release of Information. The VA will provide the Contractor with a copy of VHA Directive 1907.01(1), Health Information Management and Health Records and VHA Directive 1605.1, Privacy and Release of Information. The penalties and liabilities for the unauthorized disclosure of VA patient information mandated by the statutes and regulations mentioned above, apply to the Contractor.

4.4.4. Professional Standards for Documenting Care: Care shall be appropriately documented in medical records in accordance with standard commercial practice and

guidelines established by VHA Directive 1907.01(1) Health Information Management and Health Records:
https://www.va.gov/vhapublications/ViewPublication.asp?pub_ID=9235 and all guidelines provided by the facility.

4.4.4.1. All film interpretations will either meet or exceed established VISN standards of timeliness, accuracy, content and signature. Only NMVAHCS approved abbreviations will be used for documentation of the patient health record. All imaging exam interpretations shall either meet or exceed established standards of timeliness, accuracy, content, and signature. Only medical abbreviations from The Joint Commission's National Patient Safety Goals, current edition, will be used for documentation of the patient health record. See Section D - Attachments: D.7 – MCP 136-11: Authorized and Unauthorized Abbreviations

4.4.4.2. Interpretations shall include the following information:

4.4.4.2.1. Patient's full name, Social Security Number (SSN), date of birth, clinical history, exam case number (accession number), date of interpretation, requesting physician, exam type, interpretation, diagnostic code, and signature of contract radiologist providing interpretation.

4.4.4.2.2. Contractor will provide interpretation per routine exam to the NMVAHCS within forty-eight (48) hours after receipt of images. All STAT exams, when properly identified, will be performed and the results communicated within one (1) hour of full receipt of images and appropriate patient information. Contractor will not be held responsible for late reports caused by delays in the respective transmission of images. All exam study interpretation reports shall be transmitted to the designated NMVAHCS system.

4.4.4.2.3. All emergency exams will take precedence over all routine exams.

4.4.5. Release of Information: The VA shall maintain control of releasing any copies of patient health information or health records and will follow policies and standards as defined, but not limited to Privacy Act requirements. Contractor will not release or disclose copies of records and will refer all such requests to the Release of Information Department at the VA facility were assigned.

4.4.6. Management for Medical Records: National Archives and Records Administration record disposition requirements are found in RCS 10-1 Chapter 6, 6000 series.

4.4.7. MEDICAL RECORDS/COMPUTERIZED RECORD SYSTEMS/ DISCLOSURE/ RECORD RETENTION: All records created from these actions shall be managed per Record Control Schedule (RCS) 10-1. Close attention shall be given to disposition schedule found in RCS 10-1 Chapter 4, 4000 series (General Record Schedule 1.1). Other records included but not limited to Chapter 1, 1180.15 and 1180.23 (GRS 1.1), Chapter 5, 5020.5 (GRS 5.4). There are some specific special contracting record schedules in RCS 10-1 for individual groups of records.

- 4.4.8. MEDICAL RECORDS REQUIREMENTS: Authorities: Contractor providing treatment and healthcare services to VHA patients in the community shall comply with 45 C.F.R. Parts 160, 162, and 164 (Health Insurance Portability and Accountability Act's Privacy Rule) and any other applicable law regarding health information, including contractual obligations as outlined in the Community Care Network (CCN) contract.
- 4.4.9. Health records provided to the Contractor by the VA for the medical care and services to VA patients are covered by VHA Privacy Act system of records entitled 'Patient Medical Records-VA' 24VA10A7 at <https://www.govinfo.gov/content/pkg/FR-2020-10-02/pdf/2020-21426.pdf> Address and authority exist to disclosure these records to the Contractor for treatment purposes.
- 4.4.10. Contractor generated patient medical records are the property of the contractor and shall not be accessed, released, transferred, or destroyed except in accordance with applicable laws and regulations. Contractor shall ensure that copies of all health records pertaining to medical care and services provided to VHA patients are available for transmission and release when requested by VHA in accordance with the CCN contract, for treatment or payment purposes. As part of reimbursing the contractor for medical care and services provided to VHA patients, copies of health records may be required for payment and quality reviews.
- 4.5. Direct Patient Care: Estimated 95% the time involved in direct patient care. Contractor shall be responsible for:
- 4.5.1. Scope of Care: Contract physicians shall be responsible for providing including, but not limited to:
- 4.5.1.1. Contractor shall ensure that teleradiology equipment is maintained in good working order and in accordance with manufacturer's specifications. Records of equipment, quality control and maintenance activities shall be provided to the COR on a quarterly basis. The reports shall be provided on or before the 10th day of the month starting, the new quarter.
- 4.5.1.2. Contractor shall perform and provide to the NMVAHCS Imaging Department quarterly Peer Reviews of all interpreting Physicians.
- 4.5.1.3. Low Dose Computed Tomography (LDCT) Lung Cancer Screenings (LCS). In accordance with VHA Directive 1417 "all radiologists responsible for reading lung cancer screening CTs will have competencies specific to LCS, including the requirements for specific Lung-RADS coding used in the interpretation of LDCT for LCS screening." Documentation of this competency must be provided to the COR when a new provider enters on duty and at least annually thereafter.
- 4.5.1.4. Physicians provided in satisfaction of this contract shall be required to perform their assigned duties with regards to all admitted patients, outpatients, and eligible beneficiaries regardless of underlying disease process or physical condition of such patients or beneficiaries.

- 4.5.1.5. Contractor shall provide board certified or board eligible Radiologists to perform final teleradiology Interpretation services. Examination images captured by NMVAHCS will be transmitted to the contractor using the Digital Imaging and Communications in Medicine (DICOM) protocol and the Federal Information Processing Standard (FIPS) compliant encryption. Upon receipt of captured image and patient information from NMVAHCS the contractor shall provide an interpretation for studies. NMVAHCS personnel will inform the contractor once images are ready for viewing. These procedures will be rendered in writing once the contract is awarded.
- 4.5.1.6. Contract physician(s) shall have control of the reporting process from dictation to report sign-off/electronic signature/approval/verification. The contract physician(s) dictates, self-edits, codes, and approves/signs/verifies their reports. No transcription services shall be provided by the VAMC. Final reports are transmitted to the VA using secure HL7 messages. Report quality/content shall follow ACR standards of report structure and communication of results. Contract physician(s) shall notify clinicians of critical findings in accordance with American College of Radiology (ACR) standards, VHA and VISN alert notification policies and in a manner appropriate to the clinical urgency of the case.
- 4.5.1.7. If a serious medical condition is identified during the interpretation; the report shall be immediately called in telephonically and report to ordering physician or medical representative for their information and action. Procedures outlined in Section D – Attachments: D.5 – MCP 11-64(1): Critical Test Results and D.6 – MCP 11-81: Ordering and Reporting Test Results will be followed.
- 4.5.1.8. All STAT exams, when properly identified, will be performed and the results communicated within one (1) hour of full receipt of images and appropriate patient information. STAT exams shall be identified by the technologist verifying the study. All STAT exams must be identified at receipt of the images and paperwork by contractor. VA will not need to guarantee any minimum study volume.
- 4.5.1.9. Finalized interpretation and official signature will be immediately electronically uploaded utilizing HL7 communication into the NMVAHCS database by an attending radiologist within 24 hours of the image being transmitted to the reading site.
- 4.5.1.10. Availability to consult with a radiologist to determine the most appropriate study for the clinical scenario involved; the VA has pre-defined protocols that would be used. Fees may be charged for such consults if the consult does not result in an imaging study being performed.
- 4.5.1.11. Mechanisms must be in-place to provide notification of test results for patients receiving care in accordance with VHA Directive 1088(1), Communicating Test Results to Providers and Patients.

4.6. **ADMINISTRATIVE:** Estimated 5% of time not involved in direct patient care.

4.6.1. Must take part in medical staff functions as required to meet the standards of the Joint Commission on Accreditation of Health Care Organizations.

4.6.2. **QA/QI documentation:** The contract physician(s) shall complete the appropriate QM/PI documentation pertaining to all procedures, complications and outcome of examinations.

4.6.3. Contractor will be responsible for performing peer reviews in support of Focused Professional Practice Evaluations (FPPE) and Ongoing Professional Practice Evaluations (OPPE) on all physicians providing services supporting this contract. A minimum of 50 peer reviews are required in a period of six months following the provider's enter on duty date for the FPPE. If a provider receives an unsatisfactory result on a FPPE/OPPE by having 5% or greater 3a or 3b American College of Radiology scores the FPPE may be extended by 6 months, the provider may be restricted from interpreting certain studies, or the provider may lose credential status at the NMVAHCS. This determination is made at the discretion of the Radiology Service Chief and/or the Credentialing team. OPPEs are required on a yearly basis. OPPEs cover a period from April 1 to March 31st of the following year and require a minimum of 50 peer reviews.

4.6.4. **SYSTEM REQUIREMENTS:**

4.6.4.1. The contractor must maintain a Teleradiology system that is reliable and functional to prevent any disruption in the delivery of services. Data transmission security must be maintained at all times. Facility personnel must be notified immediately of any equipment malfunctions that would hinder the transmission of images.

4.6.4.2. Contractor warrants that any data containing patient-specific identifiers will be handled in such a way as to prevent disclosure to unauthorized parties. This includes transmission of the data (from the point at which the data leaves the VA network to the point at which the images are interpreted) as well as storage of the data by the Contractor. Contractor agrees to comply with all applicable data security laws and guidelines, as well as HIPAA and various JCAHO provisions. The contractor will provide utmost care and attention to patient data. All patients will be assured of their privacy and personal dignity in accordance with applicable laws, rules and regulations. All patient information remains the sole property of the US Government and may not be used for any purpose other than those stipulated in this agreement.

4.6.4.3. VA will act as primary custodian of the patient information (including both the image data and the report) for all purposes related to VA and Government records retention requirements.

4.6.4.4. Patient information, including images, will be retained by the Contractor only for the minimum period of time required to comply with any applicable laws, and should then be purged.

- 4.6.4.5. Upon request, Contractor will provide VA with access to information pertaining to the way in which the Contractor maintains VA patient data and the steps taken on an ongoing basis to assure the privacy and security thereof. This includes, but is not limited to, information regarding computer network architecture, configuration of firewall(s), routers, and other pieces of networking equipment, information about installed security software, and audits of patches of known security vulnerabilities. All relevant security-related patches and anti-virus updates must be installed within thirty (30) days of initial release.
- 4.6.4.6. Patient medical data transfer and storage complies with the American College of Radiology (ACR) and Digital Imaging and Communication of Medicine (DICOM) standards and meets and anticipates all current, known CONFIDENTIALITY requirements and regulations.
- 4.6.4.7. Contractor radiologists shall be available to receive phone calls from the NMVAHCS's physicians (e.g., Emergency Room STAT requests after normal operating hours) in reference to the most appropriate study to perform in the given clinical situation. If an additional study is needed, the local VAMC referring provider will be contacted by the respective Contractor's staff to communicate the need for the study.
- 4.6.4.8. Contractor shall provide a transcribed final report, which will be uploaded to the respective VAMC's hospital information system(s) and electronically signed by the interpreting radiologists.
- 4.6.4.9. Contractor shall have ability to interpret images from commercial systems. Contractor shall maintain its teleradiology equipment in good working order and in accordance with the specifications of the manufacturer. Equipment quality control shall be performed in accordance with manufacturer's specifications. Records of equipment quality control activities shall be made available to VA for review upon request.

4.7. **PERFORMANCE STANDARDS, QUALITY ASSURANCE (QA) AND QUALITY IMPROVEMENT(QI)**

- 4.7.1. **Quality Management/Quality Assurance Surveillance:** Contract personnel shall be subject to Quality Management measures, such as patient satisfaction surveys, timely completion of medical records, and Peer Reviews. Methods of Surveillance: VA Focused Provider Practice Evaluation (FPPE) and VA Ongoing Provider Practice Evaluation (OPPE). Contractor performance will be monitored by the government using the standards as outlined in this Performance Work Statement (PWS) and methods of surveillance detailed in the Quality Assurance Surveillance Plan (QASP). The QASP shall be attached to the resultant contract and shall define the methods and frequency of surveillance conducted. See Section D – Attachments: D.8 - Quality Assurance Surveillance Plan.
- 4.7.2. **Patient Complaints:** The CO will resolve complaints concerning Contractor relations with the Government employees or patients. The CO is final authority on validating complaints. In the event that The Contractor is involved and named in a validated patient complaint, the Government reserves the right to refuse acceptance of the

services of such personnel. This does not preclude refusal in the event of incidents involving physical or verbal abuse.

4.7.3. Performance Standards:

4.7.3.1. Measure: Timeliness of Reports

Performance Requirement: Prompt delivery (timeliness) of interpretations and return routine, STAT and urgent exams.

Standard: Delivery 100% of the time. Sixty (60) minutes for STAT, four (4) hours for urgent exams, forty-eight (48) hours for routine exams.

Acceptable Quality Level: 90% meet timeliness standards each month.

Surveillance Method: Review of computer logs.

Frequency: Monthly

4.7.3.2. Measure: Provider Quality Performance

Performance Requirement: All contract physician(s) shall perform in accordance with clinical standards.

Standard: Peer Review for all staff providing services under the contract. All staff (100%) shall meet Standards.

Acceptable Quality Level: 98% meet Standards

Surveillance Method: Peer review data pertinent to Radiologic Interpretations and communication of results performed for each provider working under contract.

Frequency: Surveillance will be on a quarterly basis or upon demand.

4.7.3.3. Measure: Qualifications of Key Personnel

Performance Requirement: All Contractor's physician(s) shall be board certified or board eligible in accordance with ACR Standards.

Standard: All (100%) contract physicians are board certified or board eligible.

Acceptable Quality Level: 100% No deviations accepted.

Surveillance Method: Random sampling of qualification documents.

Frequency: Quarterly

4.7.3.4. Measure: Scope of Practice/Privileging

Performance Requirement: Contract physician(s) performs within their individual scopes of practice/privileging.

Standard: All (100%) contract physician(s) perform within their scope of practice/privileges 100% of the time.

Acceptable Quality Level: 100% Contractor's physician(s) perform within their scope of practice/privileges 100% of the time.

Surveillance Method: Random sampling of records.

Frequency: Yearly

4.7.3.5. Measure: Patient Access

Performance Requirement: The Contractor shall provide Contractor's physician(s) in accordance with the schedule outlined in this PWS.

Standard: All (100%) contract physician(s) are on time and available to perform services.

Acceptable Quality Level: Contract physician(s) is available to perform services 98% of the time.

Surveillance Method: Periodic inspection or random sampling of radiology documentation and interpretations.

Frequency: Inspections will be conducted quarterly

- 4.7.3.6. Measure: Maintains licensing, registration, and certification
Performance Requirement: Updated Licensing, registration and certification shall be provided as they are renewed. Licensing and registration information kept current.
Standard: All (100%) licensing, registration(s) and certification(s) for contract physician(s) shall be provided as they are renewed. Licensing and registration information kept current.
Acceptable Quality Level: 100% licensing, registration(s) and certification(s) for Contractor's physician(s) shall be provided as they are renewed. Licensing and registration information kept current. No acceptable deviation.
Surveillance Method: Periodic inspection and random sampling.
Frequency: Yearly
- 4.7.3.7. Measure: Mandatory Training
Performance Requirement: Contractor shall complete all required training on time per facility policy.
Standard: All (100%) of required training is complete on time by Contractor's physician(s).
Acceptable Quality Level: 100% compliance.
Surveillance Method: Periodic sampling.
Frequency: Yearly
- 4.7.3.8. Measure: Privacy, Confidentiality and HIPAA
Performance Requirement: Contractor is aware of all laws, regulations, policies and procedures relating to Privacy, Confidentiality and HIPAA and complies with all standards. Zero breaches of privacy or confidentiality.
Standard: All (100%) Contractor's physician(s) comply with all laws, regulations, policies, and procedures relating to Privacy, Confidentiality and HIPAA
Acceptable Quality Level: 100% compliance.
Surveillance Method: Periodic inspection. Contractor shall provide evidence of annual training required by the facility, reports violations per VA Handbook 6500.6.
Frequency: Quarterly

4.7.4. Registration with Contractor Performance Assessment Reporting System (CPARS)

- 4.7.4.1. As prescribed in Federal Acquisition Regulation (FAR) Part 42.15, the Department of Veterans Affairs (VA) evaluates Contractor past performance on all contracts that exceed the Simplified Acquisition Threshold and shares those evaluations with other Federal Government contract specialists and procurement officials. The FAR requires that the Contractor be provided an opportunity to comment on past performance evaluations prior to each report closing. To fulfill this requirement VA uses an online database, CPARS, which is maintained by the Naval Sea Logistics Center in Portsmouth, New Hampshire. CPARS has connectivity with the

Past Performance Information Retrieval System (PPIRS) database, which is available to all Federal agencies. PPIRS is the system used to collect and retrieve performance assessment reports used in source selection determinations and completed CPARS report cards transferred to PPIRS. CPARS also includes access to the federal awardee performance and integrity information system (FAPIIS). FAPIIS is a web-enabled application accessed via CPARS for Contractor responsibility determination information.

4.7.4.2. Each Contractor whose contract award is estimated to exceed the Simplified Acquisition Threshold requires a CPARS evaluation. A government Focal Point will register your contract within thirty days after contract award and, at that time, you will receive an email message with a User ID (to be used when reviewing evaluations). Additional information regarding the evaluation process can be found at www.cpars.gov or if you have any questions, you may contact the Customer Support Desk @ DSN: 684-1690 or COMM: 207-438-1690.

4.7.4.3. For contracts with a period of one (1) year or less, the contracting officer will perform a single evaluation when the contract is complete. For contracts exceeding one year, the contracting officer will evaluate the Contractor's performance annually. Interim reports will be filed each year until the last year of the contract, when the final report will be completed. The report shall be assigned in CPARS to the Contractor's designated representative for comment. The Contractor representative will have sixty (60) days to submit any comments and re-assign the report to the CO.

4.7.4.4. Failure for the Contractor's representative to respond to the evaluation within those sixty (60) days, will result in the Government's evaluation being placed on file in the database with a statement that the Contractor failed to respond; the Contractor's representative will be "locked out" of the evaluation and may no longer send comments.

5. GOVERNMENT RESPONSIBILITIES:

5.1. VA Support Personnel, Services or Equipment:

5.1.1. The connectivity/functionality should be approved and verified by NMVAHCS prior to acceptance. NMVAHCS will be responsible for providing connectivity and infrastructure from the VA facilities to the contractor's site.

5.1.2. Electronically signed reports shall be received by the HL7 receiver and populated into the NMVAHCS Patient Record.

5.1.3. The VA uses a system that routes DICOM images. This is a one-way system that sends out DICOM images. VA will take appropriate measures with respective ISO or security personnel for compliance with security measures and connectivity. The Contractor will have no access or ability to query the VA database.

5.1.4. The VA shall make provisions for the proper maintenance and functioning of all government furnished imaging, teleradiology and associated equipment, facilities

and fixtures as may reasonably be required by Physicians to perform services hereunder.

5.1.5. The VA shall make available to Contractor copies of their internal policies, procedures, and necessary forms which physicians will require in performing their duties hereunder.

5.1.6. The VA warrants that the radiology departments meet or exceed all requirements for state licensing and any applicable accrediting or regulatory body, such as the Joint Commission.

5.1.7. The VA will use qualified personnel (i.e., registered radiologic technologists, or radiology residents) to operate equipment necessary to produce adequate images for interpretation.

5.2. Contract Administration/Performance Monitoring: After award of contract, all inquiries and correspondence relative to the administration of the contract shall be addressed to the CO and the COR.

5.2.1. CO Responsibilities: TBD

5.2.1.1. The Contracting Officer is the only person authorized to approve changes or modify any of the requirements of this contract. The Contractor shall communicate with the Contracting Officer on all matters pertaining to contract administration. Only the Contracting Officer is authorized to make commitments or issue any modification to include (but not limited to) terms affecting price, quantity or quality of performance of this contract.

5.2.1.2. The Contracting Officer shall resolve complaints concerning Contractor relations with the Government employees or patients. The Contracting Officer is final authority on validating complaints. In the event the Contractor effects any such change at the direction of any person other than the Contracting Officer without authority, no adjustment shall be made in the contract price to cover an increase in costs incurred as a result thereof.

5.2.1.3. In the event that contracted services do not meet quality and/or safety expectations, the best remedy will be implemented, to include but not limited to a targeted and time limited performance improvement plan; increased monitoring of the contracted services; consultation or training for Contractor personnel to be provided by the VA; replacement of the contract personnel and/or renegotiation of the contract terms or termination of the contract.

5.3. COR Responsibilities:

The COR for this contract is: TBD

5.3.1.1. The COR shall be the VA official responsible for verifying contract compliance. After contract award, any incidents of Contractor noncompliance as evidenced by the monitoring procedures shall be forwarded immediately to the Contracting Officer.

- 5.3.1.2. The COR will be responsible for monitoring the Contractor's performance to ensure all specifications and requirements are fulfilled. Quality Improvement data that will be collected for ongoing monitoring includes but is not limited to: enter data that may be collected.
- 5.3.1.3. The COR will maintain a record-keeping system of services by maintaining electronic folders on the service drive. The COR will review this data monthly when invoices are received and certify all invoices for payment by comparing the procedures documented on the VA record-keeping system and those on the invoices. Any evidence of the Contractor's non-compliance as evidenced by the monitoring procedures shall be forwarded immediately to the Contracting Officer.
- 5.3.1.4. The COR will review and certify monthly invoices for payment. If in the event the Contractor fails to provide the services in this contract, payments will be adjusted to compensate the Government for the difference.
- 5.3.1.5. All contract administration functions will be retained by the VA.

6. SPECIAL CONTRACT REQUIREMENTS

- 6.1. Connectivity: Service connection with VAMC shall be provided via a one-way transmission that shall be expected to transmit a minimum of 10 Gigabyte (GB) of patient images daily. The C&A requirements do not apply, and a Security Accreditation Package is not required. The connectivity/functionality should be established, approved, and verified by the VA Office of Information and Technology (OI&T) and the VAMC Department of Imaging prior to acceptance of the contract. No Government Furnished Equipment (GFE) shall be provided other than equipment necessary to make this connection. The Contractor shall be responsible for any costs associated with their connectivity to the VA network.
 - 6.1.1. The Contractor shall maintain a system that is reliable and functional in order to prevent any disruption of delivery of service. Data transmission security must be maintained at all times. OI&T, and Imaging Department personnel must be notified immediately of any equipment malfunctions that would detrimentally impact the transmission of images or reports.
 - 6.1.2. The Contractor shall demonstrate capability to interface with the VAMC Computerized Patient Record System Radiology (CPRS) Package and utilize HL7 to upload final reports directly into CPRS for immediate viewing. Faxed reports would only be used as a contingency if and when network connectivity is disrupted. No Contractor owned equipment shall be installed in the VAMC due to local security requirements. The contractor will have no direct access to the VA database.
 - 6.1.3. The Contractor, Contractor personnel, and sub-contractors shall be subject to the Federal laws, regulations, standards, and VA Directives and Handbooks regarding information and information system security as delineated in this contract.
- 6.2. Reports/Deliverables: The Contractor shall be responsible for complying with all reporting requirements established by the Contract. Contractor shall be responsible for assuring the accuracy and completeness of all reports and other documents as well as the timely

submission of each. Contractor shall comply with contract requirements regarding the appropriate reporting formats, instructions, submission timetables, and technical assistance as required.

- 6.2.1. The following are brief descriptions of required documents that must be submitted by Contractor: upon award; weekly; monthly; quarterly; annually, etc. identified throughout the PWS and is provided here as a guide for Contractor convenience. If an item is within the PWS and not listed here, the Contractor remains responsible for the delivery of the item.

What	Submit as noted	Submit To
Quality Control Plan: Description and reporting reflecting the contractor's plan for meeting of contract requirements and performance standards	Upon proposal and as frequently as indicated in the performance standards.	Contracting Officer / COR
Copy of Subcontracting Plan is required for all large businesses. Copy of Contractor Certification Statement if no-subcontracting possibilities exist.	Upon proposal and as updated	Contracting Officer
Copies of all licenses, board certifications, board eligibility (letter of residency completion, or ABR confirmation of eligibility, or current ABR candidate status printout), NPI, to include primary source verification of all licensed and certified staff	Upon proposal and upon renewal of licenses and upon renewal of option periods or change of key personnel.	Contracting Officer /COR
Certification that staffs list have been compared to OIG list	Upon proposal and upon new hires.	Contracting Officer
Proof of Indemnification and Medical Liability Insurance	Upon proposal and upon renewals.	Contracting Officer
Certificates of Completion for Cyber Security and Patient Privacy Training Courses	Before receiving an account on VA Network and annual training and new hires.	COR
Contingency plan for replacing key personnel to maintain services as required under the terms of the contract	Upon proposal and as updated	COR
Proof of LCS competency	Upon proposal, for new hires, and annually thereafter	COR
Peer reviews (FPPE/OPPE)	Within 6 months of provider enter on duty date and annually or as requested thereafter	COR

6.3. Billing:

- 6.3.1. Invoice requirements and supporting documentation: Supporting documentation and invoice must be submitted no later than the twenty (20) workdays of the following month in which services were rendered. Subsequent changes or corrections shall be

submitted by separate invoice. In addition to information required for submission of a “proper” invoice in accordance with FAR 52.212-4 (g), all invoices must include:

- 6.3.1.1. Name and Address of Contractor
- 6.3.1.2. Invoice Date and Invoice Number
- 6.3.1.3. Contract Number and Purchase/Task Order Number
- 6.3.1.4. Date of Service
- 6.3.1.5. Contract physician(s)
- 6.3.1.6. Total price

- 6.3.2. Off-site Per-Procedure: Invoice requirements and supporting documentation: Supporting documentation and invoice must be submitted no later than the 3rd workday of the following month in which services were rendered. Payment to the Contractor shall be made monthly, in arrears, upon receipt of a properly prepared invoice. Payment for services will be at the rates specified in the Schedule of Services. The Contractor shall submit invoices covering the services performed under this contract. The invoices shall contain the following information:

- 6.3.2.1. Contract Number and Purchase Order Number (if applicable)
- 6.3.2.2. Name of Beneficiary and last four digits of the Social Security Number
- 6.3.2.3. Itemized statement of services rendered by CPT Code and Rates
- 6.3.2.4. Total Price

- 6.3.3. Vendor Electronic Invoice Submission Methods: Invoices will be electronically submitted to the Tungsten website at <https://www.tungstenautomation.com/>. Tungsten direct vendor support number is 877-489-6135 for VA contracts. The VA-FSC pays all associated transaction fees for VA orders. During Implementation (technical set-up) Tungsten will confirm your Tax Payer ID Number with the VA-FSC. This process can take up to 5 business days to complete to ensure your invoice is automatically routed to your Certifying Official for approval and payment. In order to successfully submit an invoice to VA-FSC please review “How to Create an Invoice” within the how to guides. All invoices submitted through Tungsten to the VA-FSC should mirror your current submission of Invoice, with the following items required. Clarification of additional requirements should be confirmed with your Certifying Official (your CO or buyer). The VA-FSC requires specific information in compliance with the Prompt Pay Act and Business Requirements. For additional information, please contact:

Tungsten Support

Phone: 1-877-489-6135

Website: <https://www.tungstenautomation.com/>

Department of Veterans Affairs Financial Service Center

Phone: 1-877-353-9791 Email: <https://www.fsc.va.gov/einvoice.asp>

- 6.4. Payments in full/no billing VA beneficiaries: The Contractor shall accept payment for services rendered under this contract as payment in full. VA beneficiaries shall not under any circumstances be charged nor their insurance companies charged for services rendered by the Contractor, even if VA does not pay for those services. This provision shall survive the termination or ending of the contract.

- 6.4.1. To the extent that the Veteran desires services which are not a VA benefit or covered under the terms of this contract, the Contractor must notify the Veteran that there will be a charge for such service and that the VA will not be responsible for payment.
- 6.4.2. The Contractor shall not bill, charge, collect a deposit from, seek compensation, remuneration, or reimbursement from, or have any recourse against, any person or entity other than VA for services provided pursuant to this contract. It shall be considered fraudulent for the Contractor to bill other third-party insurance sources (including Medicare) for services rendered to Veteran enrollees under this contract.
- 6.5. Contractor Security Requirements (VA Handbook 6500.6). See Section D: Attachments;
 - 6.5.1. D.9 – VA Handbook 6500.6: Appendix B: VA Acquisition Regulation Solicitation Provision and Contract Clause;
 - 6.5.2. D.10 – A Handbook 6500.6: Appendix C: VA Information and Information System Security/Privacy Language for Inclusion into Contracts, as Appropriate; and
 - 6.5.3. D.11 – A Handbook 6500.6: Appendix D: Contract Rules of Behavior.

(End of Performance Work Statement)